

Elderly End of Life Care



Section I: Scenario Demographics

Scenario Title:	Elderly End Of Life Care
Date of Development:	(18/04/2018)
Target Learning Group:	☐ Juniors (PGY 1 - 2) ☒ Seniors (PGY ≥ 3) ☐ All Groups

Section II: Scenario Developers

Scenario Developer(s):	Professor Victoria Brazil, Dr Nemat Alsaba, Dr Rebecca Shaw
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Section III: Curriculum Integration

Learning Goals & Objectives	
Educational Goal:	To practice goals of care planning in elderly patients in whom further life-saving interventions may not be beneficial, and to demonstrate compassionate and informative communication with care givers.
CRM Objectives:	1. Demonstrate a sensitive end of life discussion in an ED context. 2. Coordinate staff involved in management of a dying patient, including appropriate end-of-life care.
Medical Objectives:	1. Recognize a patient in septic shock. 2. Undertake a focused assessment as to benefits and harms of interventions to support life

References

<http://onlinelibrary.wiley.com/doi/10.1111/1742-6723.12678/full>
<https://emcrit.org/emcrit/semantics-end-of-life-discussions/>
<https://www.smacc.net.au/2016/04/how-to-diagnose-dying-ashley-shreves/>

Case Summary: Brief Summary of Case Progression and Major Events

An 89-year-old patient is brought in to the ED by ambulance from their nursing home. Staff found her unresponsive and hypotensive at morning handover. She had been treated for UTI by her family physician over the last few days. Participants identify severe sepsis and realize that critical care interventions may be inappropriate. This should prompt a goals of care discussion including potential for initiating end-of-life care.



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Section IV: Scenario Script

A. Clinical Vignette: To Read Aloud at Beginning of Case

The charge nurse informs you “I just put a very unwell looking patient into resus. She’s from a nursing home facility and the paramedics think she is septic. She’s hypotensive and barely responsive. Honestly, she looks like she might be dying. Her granddaughter is on her way. I don’t think she has a known advanced care directive or code status.”

B. Scenario Cast & Realism

Patient:	☐ Computerized Mannequin	Realism:	☒ Conceptual
	☐ Mannequin		☐ Physical
	☒ Standardized Patient		☒ Emotional/Experiential
	☐ Hybrid		☐ Other:
	☐ Task Trainer		☐ N/A
		<i>Select most important dimension(s)</i>	
Confederates	Brief Description of Role		
Patient	You are essentially moribund for this scenario. You have very cool peripheries (as demonstrated with ice) and look grey and ashen. Your mouth is wide open and your eyes are staring into space. You are breathing comfortably with an occasional mild cough. You may be provided with oxygen, and/or IV fluids started. They may put your legs up and tilt the head of the bed down.		
Patient’s granddaughter	You received a call from the home that your grandmother was very unwell and had been taken to hospital. You kind of knew this day would come as it’s been a rocky past 6 months with her. She has been going downhill fast at the nursing home and no longer recognizes any of the family. She is intermittently aggressive and for the last week she hasn’t been eating or drinking anything. No one has ever brought up her end-of-life wishes with you before, but in the back of your mind you did realize that she might die soon. The GP said she had a urinary tract infection when he visited a couple of days ago and seemed to think that this is the reason she was off her food and prescribed some antibiotics. Participants will be undertaking an initial assessment recognizing the severity of the patient’s condition. They will seek information from you, notes and other sources to determine cause and likely prognosis. Ideally, they will quickly engage in a discussion regarding the patients end of life wishes.		



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	You really want to have this discussion at the patient bedside. You are a little surprised that the doctors seem to think the end is so near. You are relieved but kind of guilty that you are feeling relieved that she might die. You don't want her to suffer and feel bad about just 'letting her die'. You ask if there is anything that can be done. If the doctors ask what they think she would have wanted you are very clear that she would like to die peacefully and would not want any life support/ICU/ventilation or CPR. If the doctors ask "what do you want us to do" you are less clear and suggest that you do want things done if they will help her. The conversation ends with a plan to complete an "Allow Natural Death" Plan (also known as comfort care) to reflect and respect the wishes of the patient end of life care.
ED RN	Acts as an ED RN. Assists with bedside care of the patient. Provides initial handover.

C. Required Monitors

☒ EKG Leads/Wires	☐ Temperature Probe	☐ Central Venous Line
☒ NIBP Cuff	☐ Defibrillator Pads	☐ Capnography
☒ Pulse Oximeter	☐ Arterial Line	☐ Other:

D. Required Equipment

☒ Gloves	☒ Nasal Prongs	☐ Scalpel
☒ Stethoscope	☐ Venturi Mask	☐ Tube Thoracostomy Kit
☐ Defibrillator	☒ Non-Rebreather Mask	☐ Cricothyroidotomy Kit
☒ IV Bags/Lines	☒ Bag Valve Mask	☐ Thoracotomy Kit
☒ IV Push Medications	☒ Laryngoscope	☐ Central Line Kit
☐ PO Tabs	☐ Video Assisted Laryngoscope	☐ Arterial Line Kit
☐ Blood Products	☐ ET Tubes	☒ Other: Advanced Resuscitation Plan / Care of Dying Pathway
☐ Intraosseous Set-up	☐ LMA	☐ Other:

E. Moulage

Standardized Patient. Grey and ashen looking with cool peripheries (may simulate with cool pack or ice). A cannula is in situ having been placed by the paramedics.

F. Approximate Timing

Set-Up:	10 min	Scenario:	20 min	Debriefing:	20 min
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Section V: Patient Data and Baseline State

A. Patient Profile and History					
Patient Name: Sandra Thompson		Age: 89		Weight: 55Kg	
Gender: ☐ M ☒ F		Code Status: Requires discussion with family member, no previous code status documented.			
Chief Complaint: Found unresponsive and hypotensive at nursing home morning rounds.					
History of Presenting Illness: Has been treated for UTI by GP over the last few days					
Past Medical History:	Hypertension		Medications:	Acetaminophen	
	Osteoporosis and previous left wrist fracture			Citalopram	
	UTI			Amlodipine	
	Depression			Amitriptyline at night	
Allergies: No Known Drug Allergies					
Social History: In nursing home facility. Was admitted there 6 months ago after a prolonged admission to hospital with delirium and sepsis, on a background of progressive cognitive decline.					
Family History: N/A					
Review of Systems:	CNS:	Patient is non communicative			
	HEENT:	Patient is non communicative			
	CVS:	Patient is non communicative			
	RESP:	Patient is non communicative			
	GI:	Patient is non communicative			
	GU:	Patient is non communicative			
	MSK:	Patient is non communicative	INT:	Patient is non communicative	
B. Baseline Simulator State and Physical Exam					
☐ No Monitor Display		☐ Monitor On, no data displayed		☒ Monitor on Standard Display	
HR: 90/min	BP: 62/40	RR: 28/min	O ₂ SAT: 86% RA		
Rhythm: sinus rhythm, broad complex	T:35.1 °C	Glucose: 2.1mmol/L	GCS: 12 (E4 V2 M6)		
General Status: Ashen and unwell looking. Minimally responsive					
CNS:	Lethargic and moaning. PEARL 3mm				
HEENT:	Nil				
CVS:	Normal heart sounds. Wide complex sinus rhythm on ECG				
RESP:	Shallow respirations. Decreased air entry to the right base with inspiratory crackles.				
ABDO:	Soft and non-tender. BS present				
GU:	Nil				
MSK:	Nil	SKIN:	Cool peripheries		

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Section VI: Scenario Progression

Scenario States, Modifiers and Triggers			
Patient State	Patient Status	Learner Actions, Modifiers & Triggers to Move to Next State	
1. Baseline State Rhythm: sinus, broad complex HR: 90/min BP: 62 /40 RR: 28/min O ₂ SAT: 86 % T: 35.1°C	Drowsy, moaning, cool peripheries. IV access already in place.	<u>Learner Actions</u> - ☐ Collateral history and examination - ☐ Applies monitors - ☐ Ask about code/DNR status and attempt to speak to family - ☐ ECG - ☐ Check glucose (2.6) - ☐ Labs, including critical VBG - ☐ May give IV fluids, O ₂ , dextrose, empiric calcium gluconate	<u>Modifiers</u> <i>Changes to patient condition based on learner action</i> - O ₂ applied → sats 89% - Dextrose given → glucose 8.2 <u>Triggers</u> <i>For progression to next state</i> - All actions complete or 5 mins into case → ED RN prompts that relative has arrived → 2. Worsening Hemodynamics
2. Worsening hemodynamics Rhythm: Sinus, wide complex HR: 72/min BP: 60/40 RR: 10/min O ₂ SAT: 82 % T: 35.1°C	As above. Family member discussion – “How is she doing?” “What kind of treatment does she need?” “Can I stay with her?”	<u>Learner Actions</u> - ☐ End of life discussion with family member/substitute decision maker - ☐ Call social worker	<u>Modifiers</u> - If participants attempt active management family member to state “she wouldn’t want all of this” - If participants continue to attempt active management ED RN to prompt: “I think you better talk with the family member, it seems like she is really uncomfortable with what we are doing here” <u>Triggers</u> - Adequate discussion with family member completed → 3. End of Life Care Planning
3. End of Life Care Planning	Patient status as above	<u>Learner Actions</u> - ☐ Agreement on plans - ☐ Communication of plans to team - ☐ Prescribe medications for comfort (opioids, benzodiazepines, fan +/- glycopyrrolate)	<u>Modifiers</u> - If no meds ordered for comfort, ED RN to prompt “Can you write up something for the pain and secretions?” End Case PRN



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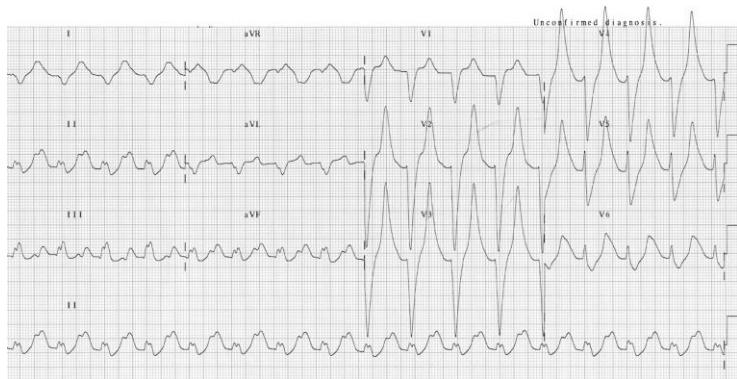
Section VII: Supporting Documents, Laboratory Results, & Multimedia

Laboratory Results

Na: 129	K: 7.8	Cl: 104	HCO ₃ : 13	BUN: 15.3	Cr: 259	Glu: 2.2
Ca:	Mg:	PO ₄ :	Albumin: 30			
VBG	pH: 7.1	PCO ₂ : 61	PO ₂ : 70	HCO ₃ : 13	Lactate: 9.0	
WBC: 15.1	Hg: 120	Hct:	Plt: 150			

Images (ECGs, CXRs, etc.)

ECG:



ECG source: <https://litfl.com/hyperkalaemia-ecg-library/>

CXR:



CXR source: <https://emrems.com/2013/01/30/how-to-you-tell-its-a-right-middle-lobe-infiltrate/>

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Section VIII: Debriefing Guide

General Debriefing Plan			
☐ Individual	☒ Group	☐ With Video	☒ Without Video
Objectives			
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Sample Questions for Debriefing			
1. What factors influenced the decision that this patient was dying? 2. What were the priorities for management in this dying patient? 3. How did making the decision this patient was dying affect the team? Were all members of the team on the same page? 4. How did it feel having an end of life discussion with the patients relative? What strategies did you utilize to communicate with this relative in a compassionate manner?			
Key Moments			
1. Recognition of dying patient 2. Discussion with relative regarding ceiling of care and end of life care. 3. Implementation of end of life care			